

إسم المريض رابعياً : زينب ابراهيم محمد حامد

المحافظة : القاهرة

الحى / المدينة : السيده زينب

العنوان : 0

تاريخ تسجيل الإستمارة : 23-09-2026

رقم البطاقة : 28305090100808 رقم التلفون : 01101806119

العمر(تاريخ الميلاد) : 09-05-1983 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمى وتلطيفى [6579] **قسم العلاج :** الاورام

التعليق على التشخيص : -----

GR1-E
جورستان
روستاییلدانی
لیدی حساب
جورستان
صحة الامانات المرفعة
اللجنة و مدير

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

تعتمد صحة الاياماب المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس

وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- /s ----- /s ----- /s.l ----- /s.l

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

ملاحظة:

ملأ خطاك : ---

قسم :

نفقات :

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

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Name: زينب ابراهيم محمد حامد National ID: 28305090100808	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٢٢١
٢٠٢٥
السيد مع مدي
د. / هادي السيد
P / خالد

٢٠٢٥
٢٠٢٥

١١-١٨-٦١١٩

02/04/2026

13

MDT FORM

Name:	Zenab Ibrahim Mohamed Hamed	MRN:	4487
Age:	43 Y	ID:	3331/2015
Menopausal Sta	PERIM	Phone:	01101806119
NRS SCORE:		Marital Status:	MARRIED
BMI:		Family History:	
CO Morbidity:	M:FREE S:APPECTOMY	Reason Of MDT:	REMDT

Clinical Data:

BX:	RT, IDC	R	L
ER:	8/8	PR:	8/8
HER2:	+3	K167:	25

Sonomammography:

US: 12 O'CLOCK LESION 3*1.7CM NO SIGNIFICANT LN
MRI AFTER DOSTINEX: TWO ILL DEFINED LESIONS 2.4*1.8*1.7CM
8.2*1.5*1CM
FEW ENLARGED LN
CLINICALLY :T2 N1 PALPABLE ALN

Mets Workup:

CTCAP: D10 LESION
BONE SCAN :FREE

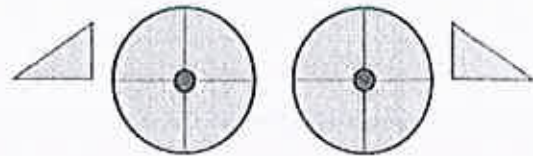
History:

Recommendations:

FOR POSSIBILITY NACT(FERTILITY PRESERVATION AS PT WISHES) CLIP
INSERTION THEN RT,BCS+SLN ACCORDING TO RESPONSE

MDT Outcome & Plan

CLIPPING BRAKITING +TCHP 6 CYCLE +ZOLADEX FOR QFS THEN BCS+ AFTER
NEW BSM



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr. /	Dr. /	Dr. /			

Patient Name: Zainab Ibrahim Mohamed Hamed	Date : 23/11/2025
Exam : CT CAP+C (CT Chest Abd, Pelvis)	Age : 42 Year(s)
Medical ID: 3331/2025	Pat. ID: 132296

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, with triphasic study of the ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Reported case of right breast cancer

(I) Chest:

- Right breast small ill defined nodules measuring 14x 11 mm 13 x 9 mm for sonomammographic correlation.
- Multiple enlarged right axillary level I & II lymph nodes, the largest measures 15 x 10 mm.
- Patent central air ways with well inflated both lungs.
- Both lung fields are clear showing no parenchymal nodules, cavitory lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is average in size showing homogenous CT density with no enhancing focal lesions
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.

- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.

- **(III) Examined skeleton:**

- D10 small lytic lesion
- No vertebral structural collapse or wedging.

OPINION:

- No pulmonary or, hepatic metastatic deposits
- D10 small lytic lesion likely benign ,

Dr.Reham Khedr , MD

Signature reham kheder
24/11/2025 1:35 PM

Signature reham kheder
24/11/2025 1:46 PM

Patient Name : الأستاذ / زينب ابراهيم محمد	Request No. : 13008785
Patient No. : 28305090100808	Request Date : 25/10/2025
Path No. : 3255/25	Verification Date : 30/10/2025
Sex : Female	Financial Category : جوميتاف روسي - مبادره
Age : 42 Y	Department : Out Patient
Referreing Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data

Right breast 12 o'clock mass.

Nature of specimen

Tru cut needle biopsy collected on 23/10/2025.

Gross examination

Mutiple(3)grey white fragmented tissue cores ranging in size from 0.8 up to 1.5 cm, totally submitted .

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against: HER2/NEU (4B5), KI67 (30-9), PR (1E2), ER (SP1).

Microscopic examination

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation + nuclear pleomorphism+ mitosis): (2+2+2) Grade 2
- * DCIS: Focal , intermediate grade (papillary & cribriform sutypes) 2 %
- * Lobular intra epithelial neoplasia : Absent
- * Angioinvasion: Positive
- * Microcalcification : Absent
- * Perineural invasion: Absent
- * IPT (Positive control showed appropriate reactivity) :
- * ER: Strong nuclear reaction in 98 % tumor cells (Allred score 8/8)
- * PR: Strong nuclear reaction in 98% tumor cells (Allred score 8/8)
- * HER2: Strong complete membrane reaction in >10 % of tumor cells (3+)
- * Ki 67: 25 %

Verified by :

Dr.Heba Hasan

Printing Date : 13/11/2025 10:38:46

Page 1 of 3

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / زينب ابراهيم محمد
Patient No. : 28305090100808
Path No. : 3255/25
Sex : Female
Age : 42 Y
Referring Doc :

Request No. : 13008785
Request Date : 25/10/2025
Verification Date : 30/10/2025
Financial Category : جوستاف روسي - مبانره
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Diagnosis

RIGHT BREAST MASS, TRU CUT BIOPSY :

- Invasive duct carcinoma Grade 2 (tubular differentiation 2, pleomorphism 2, mitosis 2)
(according to Nottingham grading system of breast cancer) ,

IPT:

- * ER: Positive (score 8/8).
- * PR: Positive (score 8/8).
- * HER2: Positive (score +3)
- * Ki67: 25 %
- * Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim
Dr. Heba El-Shafeey

Verified by :

Dr. Heba Hasan

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Page 2 of 3

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name : Zainab Ibrahim Mohammed Hamid
Branch : Dokki
Ref.Physician Prof : Gostaf Roseh Hospital

Date : 19-11-2025 02:05
Medical No. : 253647
Ins.Company : تعاقد مستشفيات

Tc^{99m} MDPSKELETAL SCINTIGRAPHY

History

- A 42-year-old lady, with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine



Patient Name: Zainab Ibrahim Mohamed Hamed	Date : 14/10/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 42 Year(s)
Medical ID: 10838	Pat. ID: 132296

Bilateral BREAST ULTRASONOGRAPHY REVEALED:

Indication: Palpable right Breast mass in lactating woman

- Heterogenous (mixed fibroglandular and fatty) echo-texture of both breasts.
- there is hypoechoic non circumscribed mass at 12 o'clock position at B zone, it shows irregular speculated outline with internal vascularity . It measures about 3 x 1.7 cm.
- prominent retroareolar ducts
- Normal skin and nipple.
- No detected suspicious solid or cystic masses in the left breast.
- No significant axillary lymph nodes .

CONCLUSION:

- Highly suspicious looking right breast mass, Bi-RADS 4c category.

MUCH OBLIGED
DR/farah elshobaky

ECHO REPORT

NAME Abdullahi Ali

DATE: 3 / 6 / 2026

ID NUMBER

AGH

EF	65%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.8	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVEDS	2.5	2-4cm	LA	3.6	2-4cm
FS	30%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions

LV Wall Thickness ~ 1

LV Diastolic Function

SWMA 270 R33A

LA and Ao Root Dimensions

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

DR

23/11/16

جمهورية مصر العربية
بطاقة تحقيق الشخصية



زينب
ابراهيم محمد حامد
١٢ ارب الطالقنى
السيدة زينب - القاهرة



٢٨٣.٥٥.٩.٠١.٠٠٨.٠٨

KJ8293397



٢٠٢٤/٠١ ٢٨٣.٥٥.٩.٠١.٠٠٨.٠٨
حصله على دبلوم تجاره



اننى
مسلمة
حسن عدائى حنفى
متروجة
البطاقة سارية حتى ٢٠٣٧/٠١/٢٧



Patient's Name:

Zainab Ibrahim Mohammed Hamid

Date:

2026-09-16

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Pathologically proven Right breast cancer, post NACT assessment study.

Findings:

Right breast:

- Scattered fibroglandular densities. (ACR b)
- Upper central, architectural distortion with tissue marker within. No suspicious microcalcifications.
- Ultrasound upper central, 12 o'clock, area of granulation tissue with absent internal vascularity and surrounding distortion, with tissue marker within.
- 1 O'clock middle third two small simple cysts, largest measured 7 mm in diameter.
- Retro-areolar dilated ducts reaching 4 mm in caliber with clear anechoic content.
- Normal skin/nipple/areola complex.
- Indeterminate level 1 axillary two lymph nodes showing globular shape, the largest measured 9 x 7 mm.

Left breast:

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses asymmetry, distortion or suspicious microcalcifications.
- Ultrasound demonstrated heterogeneous breast with mottled appearance with no solid or cystic lesions.
- Retro-areolar dilated ducts reaching 4 mm in caliber with clear anechoic content.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 6): complete radiological response of the previously reported pathologically proven malignant process yet with indeterminate ipsilateral axillary lymph nodes. UIQ two small simple cysts

Left breast (BIRADS 2): Retro-areolar clear duct ectasia

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations

BIRADS 0: Non-informative study.

Suspected Cancer Risk

Post NACT

No MDI

Patient's Name:

Zainab Ibrahim Mohammed Hamid

Date:

2026-08-27

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

I- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

❖ No hepatic or pulmonary metastatic lesions.

Dr.Assem Ahmed



جمهورية مصر العربية
وزارة الصحة و السكان
المجالس الطبية المتخصصة
الشبكة القومية لعلاج المواطنين على نفقة الدولة

خطاب إدارى

الرقم القومى	٢٨٣٠٥٠٩٠١٠٠٨٠٨
إسم المواطن	زيثب ابراهيم محمد حامد
رقم الطلب	٦٦٦٢٢٤٨٨
تاريخ اللجنة	٢٠٢٦.٠٧.٠٩
الحالة	خطاب إدارى
نص الخطاب	٢٢٧١٤ / ٣٩٢٥ / ٢٨٦٦٤
ايكو + مقطعية حديثة للصنر + سبب تاخر الحالة في بدء العلاج — التشخيص 2025-11	
جهة الإرسال	جوستاف روسي الترنشيو نال - مصر (EPE) *

د/ محمد عقاد
الإدارة العامة للمجالس الطبية المتخصصة
مدير عام

تحريراً في ٢٠٢٦-٠٧-٠٩
٢٢٧١٤ / ٣٩٢٥ / ٢٨٦٦٤